

Graham Duncan couldn't focus on much except the pain.

The 62-year old was on the floor of his west-end Toronto home with a broken femur. His wife Laurel was nearby, waiting for an ambulance to come help. Time blurred, but Duncan knew it had been a while since he dismounted terribly wrong from his bike; since his neighbour all but carried him to the door and since, in his initial surge of adrenaline, he scooted upstairs.

Now that adrenaline had worn off and moving again seemed impossible – certainly not downstairs, ruling out a car ride to the nearest hospital.

"If I could have gotten off the floor, I would have," Duncan said.

The only thing he could do that September 2024 evening was wait for paramedics to arrive. But hours ticked by – one, two, three, then four.

Laurel called 9-1-1 three times, in those hours: her husband, who also had cancer, was in worsening agony and immobile on the floor. When would the paramedics be able to help?

- *SPECIAL BUILD: Laurel, 8:52 p.m.: "Well, I already made a call - just after 6:00 ... (either cut or beep out personal stuff like address?) ... my husband fell off his bicycle earlier today and he's in an extreme amount of pain in his leg. He's unable to stand up...*
- *Dispatch: "Is he awake?"*
- *Laurel: "He's awake, yes."*
- *Dispatch: "Is he breathing?"*
- *Laurel: "He's conscious. He's breathing. He's awake –"*
- *Dispatch: "Is he responding normally?"*
- *Laurel: "Yes. Yes. Just screaming in pain sometimes."*

This is a problem that has besieged Toronto's paramedic service for years, and played out in plain sight of decision-makers. Overstrain on Toronto emergency medical system has left patients waiting, sometimes for hours, in painful, uncomfortable and even dangerous situations.

Officials have acknowledged that something needs to change. In June 2025, days after bystanders waited on hold with 9-1-1 for nearly seven minutes after a 15-year-old boy was shot and lay waiting for help¹, Toronto Mayor Olivia Chow told reporters there would be a \$52-million investment over the coming years to boost staffing and hasten responses:

"Torontonians deserve a paramedic service that is quick to respond in an emergency because people cannot wait."

But that hasn't happened.

A Star investigation, reviewing detailed records on every ambulance response across Toronto between May 1, 2025 and May 1, 2026 – more than 330,000 calls – reveals an alarming portrait of Torontonians stuck waiting after calling help during emergencies.

They waited during suspected heart attacks and strokes: the kind of emergencies where doctors say every minute waiting for help matters, with tissues starved of oxygen dying bit by

1

https://www.thestar.com/news/gta/paramedics-reviewing-response-time-in-fatal-shooting-of-teen-after-reports-911-callers-were-put/article_8f4d418f-deef-4524-96d5-8f155f0e8d34.html

bit, worsening a person's long-term outcomes and odds of survival. They waited after reporting breathing troubles, traumatic injuries, choking and seizures.

More often than not, in the most serious cases, ambulance responses failed to hit the service's own internal response targets.

The data analysed by the Star shows only 27 per cent of calls assigned the highest severity level, of imminently life threatening conditions, met internal response targets; among potentially life-threatening calls, only 37 per cent hit targets.

A Star investigation found more than 4,600 of these highest priority calls – known as “purple” and “red” calls – had wait times over 30 minutes. More than 1,100 stretched above an hour.

The majority of these long-wait calls, the paramedic service stressed in response to the Star's findings, are calls like Duncan's – where the initial priority level assigned to the call did not reflect the patient's actual condition upon arrival, requiring it to be bumped up in urgency.

But Toronto Paramedic Services said it, too, was concerned about patients' health outcomes being impacted in these cases as waits stretch out. “Some high-priority calls experience delays, particularly during periods when emergency call demand is elevated and ambulance availability is low,” a spokesperson said, citing growing call demand, staffing issues, long in-hospital wait times to offload patients and travel conditions among pressures at play.

TPS continued to push for “system-wide solutions” to improve ambulance availability, the service said – “ensuring patients receive the most appropriate care as quickly as possible.”

Still, to patients and paramedics alike, these waits are signals of a broken system.

Nearly three hours after she first called 9-1-1, Laurel was again on the phone with the dispatchers. The call-taker didn't mince words.

Dispatch: “Unfortunately, it's really busy in the city and our ambulances are responding to life-or-death emergencies, so at this time we don't have anyone to respond.”

When responses bleed long, muscle dies and patients suffer

VG: Special build begins here maybe? Bubbles could detail specific incidents like:

Dec. 6, 2025, 12:47 a.m., Orange

A father of two experiences sharp, sudden chest pain and tightness at home while his children are asleep. His wait for paramedics is one hour and 10 minutes. “Is that really something that should happen in a city like Toronto?” Noah Waksman wonders.

April 24, 2026, 8:45 a.m., Red upgraded to Purple

Someone is choking in northern Scarborough. The call is initially deemed as potentially life-threatening, then upgraded to imminently life-threatening. An ambulance is dispatched within three minutes, but arrives at the scene more than 20 minutes after the initial call.

Jan. 20, 2026, 10:09 a.m., Red

A patient is triaged as having a serious STEMI heart attack, categorized as “unstable.” Paramedics arrive after 37 minutes and 48 seconds – one of three calls triaged as unstable and potentially life-threatening STEMIs in a year with response times above 20 minutes.

Dec. 31, 2025, 9:30 a.m., Red

A patient is triaged as having a potential CVA stroke, and being in potentially life-threatening shape. An advanced care team is initially assigned, before being altered to send a primary care team. The response in the M9L area of Toronto took one hour and six minutes.

Nov. 1, 2025, 1:02 p.m., Orange upgraded to Purple

A patient initially believed to be in urgent condition has suffered a hemorrhage or laceration. The patient, in the M6A area, winds up being in imminently life-threatening shape. The wait for the first of three ambulances assigned to this call was 33 minutes and 42 seconds.

Dec. 4, 2025, 10:28 AM, Orange

A man fell down a flight of stairs in an old Cabbagetown house during an on-site photoshoot, and sustained painful injuries. His wait time: one hour and 47 minutes. "It was a very confusing, stressful, anxious time," colleague Jonathan Mutch recalled to the Star.

Dec. 2, 2025, 7:28 a.m., Orange upgraded to Red

A woman in her 50s is stabbed by a man who police allege waited for her outside of a business, and left with life-threatening injuries. An ambulance took 21 minutes, 59 seconds.

Dec. 12, 2025, 9:44 p.m., Yellow

Someone suffers a fall in Scarborough's M1R area, triaged to be the lowest level of acuity on the ambulance dispatch triage scale. So they wait, and wait, and wait. The paramedics finally arrive to help seven hours and 44 minutes after the 9-1-1 call was initially placed.

Nov. 17, 2025, 12:51 p.m., Orange upgraded to Purple

A call for suspected psychiatric issues – ranging from abnormal behaviour to suicide attempts – in the Church-Wellesley area downtown is initially triaged as urgent, but at some point after the call, becomes immediately life-threatening. Paramedics are assigned to it after 24 minutes, and the first vehicle arrives 48 minutes and 26 seconds after the initial call.

Inside Toronto hospitals, emergency physicians know that every minute matters.

When it comes to a stroke, for example, "time is brain," says Dr. Garrick Mok, a physician with downtown St. Michael's Hospital's emergency department. Every minute that ticks past, more brain cells can die, worsening the injury to a patient and making it harder for them to recover and rehabilitate.

The same goes for cardiac arrest patients, he said, where the heart is starved of oxygen-rich blood.

"The longer that the muscle is without oxygen and blood supply, the more it's either dying or not getting the blood supply it needs ... that area of the heart might not actually recover."

At their fastest, Toronto paramedics have responded to stroke cases in less than a minute. But among just one type of stroke call captured in the Star's database – called CVAs – 261 red and purple level calls took more than 30 minutes, and 68 took more than an hour.

The year of ambulance responses analysed by the Star includes a call for a severe heart attack (known as a STEMI), categorized as "unstable," which took 37 minutes and 48

seconds for an ambulance to arrive. Three “unstable” STEMI calls took upwards of 20 minutes, and were each categorized upon final assessment as a “red” level priority.

The data also includes strokes with response times above four hours. The most egregious case, in December 2025, was also assigned a red-level priority, but was initially triaged as a much less severe yellow case. The response time was four hours and 36 minutes, the records show. Of the stroke calls, 193 calls of varying severity levels took upwards of an hour between the first 911 call and first ambulance arriving on scene, according to the data.

The response time captured in the data represents the time between the first 9-11 call and the ambulance arriving on scene. It does not factor in the time it takes to reach a patient if they are inside a building, to assess a patient on-site, or to transport them to hospital.

For strokes, Mok said, in-hospital tests are required to determine whether someone can safely receive thrombolytic medication to dissolve a clot and restore oxygen to their brain during a stroke – or if they have a brain bleed, meaning that treatment could kill them.

“You need to get a CT scan,” Mok said.

“That’s why all that timing is so important ... you’ve got to get picked up, you’ve got to get brought over, you’ve got to get transferred to our emergency department, you’ve got to get a CT, you’ve got to look at the CT in real time and then we’ve got to decide, are we going to give you this medication?”

Sahil Gupta, another emergency physician in the St. Michael’s ER, said intervention for a patient having a stroke should be done within 3 to 4.5 hours of the stroke, meaning a patient must be brought to hospital, registered, medically assessed and cleared for treatment.

But both Gupta and Mok, as with other medical experts who spoke to the Star, stressed that paramedics’ response times weren’t solely an EMS problem – they were, in their view, a symptom of the broader healthcare system struggling against mounting pressure.

Data visualization - how many calls go beyond the targeted response times?

How things got so bad

The deterioration of ambulance response times in Toronto has been a slow march. Two years ago, city hall’s top auditor pointed to a problem that had worsened markedly since at least 2019². In that time, auditor general Tara Anderson wrote in a public report, there had been “significant” jumps in how often, and for how long, there was low availability of staffed ambulances across Toronto – rising from 1,300 incidents in 2019 to 6,800 incidents in 2023.

The most critical cases, where not a single staffed ambulance is available across the city, had also jumped – from 29 incidents in 2019 to more than 1,200 four years later, she wrote.

Anderson pointed to festering problems from long handovers in hospital to a growing reliance on “full-shift overtime,”³ which had risen 169 per cent in those same four years, adding to the risk of stress-related injuries and staff turnover for frontline workers.

² <https://www.toronto.ca/legdocs/mmis/2024/au/bgrd/backgroundfile-247124.pdf> Pg. 3

³ <https://www.toronto.ca/legdocs/mmis/2024/au/bgrd/backgroundfile-247124.pdf> Pg. 4

“Timely ambulance emergency response is critical, particularly for people with medical issues requiring urgent attention at the hospital,” the AG’s office wrote. “Every second matters for ambulance emergency response, particularly when it is a matter of life or death.”

Those who spoke to the Star for this story – including physicians, patients and paramedics themselves – stressed that protracted waits for ambulances are a consequence of pressures on the healthcare system rather than failings of individual medics, or the service on its own.

Ambulance demand is rising for a constellation of entangled reasons, experts say: Baby Boomers are getting older, requiring more urgent care. At the same time, more and more Torontonians do not have access to a family doctor, so their needs spill over into the emergency system. Other social crises like worsening drug toxicity only add fuel to the fire.

Staffing, over time, has not kept pace. Anderson’s 2024 report pointed out that it would be “challenging” to make any improvements to ambulance response times without more people on the payroll. When Chow vowed improvements to the service last summer, it came alongside the long-awaited promise of a multi-year staffing plan, at a cost of \$52 million.

But gains, each year, are cannibalized by departures. In 2025, Toronto hired 167 new paramedics, but lost 97, according to records obtained via freedom of information legislation. A similar story played out in 2024, when 162 medics were hired and 90 left the service.

In many cases, front-line sources tell the Star, the departures are not fueled by retirements but instead by early- or mid-career paramedics, burnt-out by the relentless pace. Some leave for similar jobs in nearby jurisdictions that are not under the same strain as Toronto.

“We were so behind already in staffing that we’re constantly playing catch-up,” one paramedic lamented. “It’s almost impossible to hire enough medics, to both cover your increases in call volumes and backfill all of the gaps that you had for years of austerity.”

The Star spoke with NUMBER paramedics in Toronto in reporting this article. The Star has agreed not to publish these paramedics’ names, as each feared speaking critically about their employer could endanger their jobs.

Not all paramedics are also considered to be actively available for emergency responses.

While numbers fluctuate by the day, data obtained by the Star shows that at the end of last year, more than five per cent of the overall workforce – 79 medics out of 1,551 – were considered inactive and unavailable for frontline deployment. More and more, medics are also getting hurt on the job, with 543 “lost time” injuries reported to workplace insurance within TPS last year⁴, having inched up steadily over the years from 418 injuries in 2021.

More than double the number of injuries were reported by paramedics versus fire services.

The paramedic service says its multi-year staffing plan is hitting its targets so far. In total, that plan involves adding 362 new roles between 2025 and 2028 – 347 of those being frontline workers, including 331 paramedics and 16 supervisors – to “enhance emergency response capacity.” Since 2025, it has added 180 “net” new paramedics, the agency said.

But they acknowledged that departures are meanwhile paring numbers back. “Like many healthcare organizations, Toronto Paramedic Services operates in a competitive labour

⁴ <https://www.toronto.ca/legdocs/mmis/2026/gg/bgrd/backgroundfile-287106.pdf>

market and continues to experience retirements and other workforce attrition. As a result, hiring efforts have been adjusted to address turnover and support planned service growth.”

To better support recruitment and retention, the statement pointed to “a range of initiatives” being undertaken, including targeted strategies developed alongside workers’ unions.

On the ground-level, paramedics still feel their teams’ energy spread thin.

There used to be time to decompress after calls, one medic said, especially the “hot calls” that grappled with harder, heavier situations. “Now, it’s kind of turning into automatons or robots. You’re just a body in a seat attending the next call, next call, next call,” one said.

The service sees offload at hospitals as a key pinch point for improving its response times. In its statement, it said 60 per cent of the average paramedics’ shift time is spent in hospital on a daily basis.

“In-hospital wait times for paramedics is the most significant challenge negatively impacting ambulance availability in the community,” its statement to the Star said.

But to some paramedics, cutting down time in hospital just means asking the existing workforce to do more than they’re already juggling – at a time when many frontline workers are already feeling stretched to their capacity, ricocheting around the city from call to call.

One especially demoralizing kind of shift, one longtime paramedic told the Star, was when your ambulance was dispatched to a lower-level priority call, you drove to the scene, and shortly before arriving, dispatch got in touch to say you would be re-routed to something more urgent. Sometimes, the paramedic said, you could be as close as the parking lot.

What that meant was leaving the first patient – potentially a person who was in pain or distress, if not as life-threatening as another emergency – to wait for the next available ambulance, sometimes after they’d already been waiting for some time for help to arrive.

And it was time spent on the road that didn’t wind up helping anyone, that paramedic lamented. “Who is keeping track of the wasted minute times of driving around in circles?”

The multi-hour calls

Duncan’s wife, Laurel Purvis, first dialed 9-11 at 6:11 p.m, about 45 minutes after Duncan initially fell, as he tried to see if the pain dissipated and he could stand up on his own first.

The dispatcher triaged his situation – a man with a cancer diagnosis in a second-floor unit, with an injury from a bike accident but no visible deformity or bleeding. The location of his injury was determined to be non-dangerous. Duncan’s case, overall, was deemed low-acuity – despite the pain he was reporting. So, it was put in the queue, and the two of them waited.

Purvis called 9-11 back at 7:08 p.m., telling dispatchers that Duncan was in worsening pain.

“I just wondered if there was any sense of when they might come,” Purvis asked.

“Unfortunately, Laurel, there is still a delay,” dispatch replied. “We don’t have any available ambulances just yet, but as soon as we have one available, we’ll be sending it to you, ok?”

Again, they waited. Purvis called a third time – this time, at 8:52, after nearly three hours. Still, they were told to hold tight. Toronto's paramedic service wasn't yet available to help.

"I do understand that I was safe in my apartment and not unconscious," Duncan recalled of that long evening. But even after self-medicating to try to dull the injury, every hour that ticked past was a painful one. "According to the specialist who operated, a bone break through cancer – her phrase – it's the ultimate pain. So yeah, I was not doing too well."

An ambulance was finally assigned to the call at 10:09 p.m., arriving by 10:22, records show.

That's when it became clear to the paramedics on the scene: this was a more serious case than the dispatch triage level had suggested. In fact, it was so serious that they needed different paramedics, ones with the ability to administer potent IV painkillers in order to extricate Duncan from the upstairs unit without causing him more undue pain.

A second ambulance was assigned at 10:34 p.m. and arrived on the scene within minutes.

Experts warn that slow medical intervention can turn low- or moderate emergencies into more pernicious ones. Over the 12 months of data analyzed by the Star, falls were one of the most common reasons for a 9-11 call. For thousands of them, response times stretched beyond the targeted times. In 140 cases, the response time was three hours or longer.

"Lying on a hard surface where muscles and tissue are pressed hard by one's body weight against a hard surface can quickly lead to muscle and tissue breakdown – and what we call rhabdomyolysis – this condition and other associated issues can lead to a deteriorating health situation for a frail older person," said Dr. Samir Sinha, a geriatrician who works in Toronto's Sinai Health System and the University Health Network.

The muscle breakdown can release toxins causing kidney failure, Sinha said. Fluid could build up in their lungs, or pressure sores on their skin. One distressing reality is a person who cannot move for multiple hours may have to wait in their own urine, he told the Star.

After three hours on the ground, Sinha said hospitalization becomes very likely – not solely because of the initial injury, but because of the conditions that can develop while immobile.

The paramedic service, in a statement, said the majority of especially urgent calls with waits above 30 minutes – the red and purple ones – are calls like Duncan's, where the initial priority did not reflect the patient's actual condition upon arrival and was bumped up.

JMc: We previously wrote: "Looking at calls across the Star's database where the priority level changed sometime between initial dispatch and the end of the emergency response, the vast majority were cases where the initial priority was too low and upgraded, versus too high and downgraded." Is it also accurate that of the calls that took 30+ minutes, most were upgraded in priority? If so, I think we should make that point here. Include the context for reasons why paramedics said that could be, but also (if accurate) that they couldn't rule out the condition worsened because the patient had to wait so long.

Paramedics who spoke to the Star say there are many reasons this can happen, from a patient's medical condition worsening during their wait for an ambulance to a caller downplaying their symptoms initially or having trouble properly communicating them to the 9-1-1 operator, including cases involving language barriers or mental health challenges.

Still, the service – in its statement – said it was concerned about patients' health outcomes being impacted in these cases. “Some high-priority calls experience delays, particularly during periods when emergency call demand is elevated and ambulance availability is low,” wrote Dineen Robinson, acting commander of policy, projects and process improvement – also citing staffing, hospital offload times, and travel conditions among pressures at play.

Pressure rises, response times erode

Over the course of the last year, the ambulance data obtained by the Star shows that the service has made a small improvement on average response times for purple calls. From May 2025 to the end of April 2026, response times dropped to an average of 469 seconds (a little less than eight minutes) from an average of 538 seconds (about nine minutes).

But every other level of urgency stagnated or worsened over this same time period.

The paramedic service, asked about the Star's findings, said it measures call response outcomes differently – counting calls more simply as “high” or “low” priority. Last year, the service said its response time to 90 per cent of high priority calls was 14.5 minutes, which has come down in 2026. But the tradeoff is that low priority call response times have gotten worse – by 12.2 per cent, compared to the 9.7 per cent improvement for high priorities.

“These trends are consistent with Toronto Paramedic Services' efforts to improve resource availability for high-priority emergencies and direct available paramedic resources to critically ill or injured patients,” the service said, calling the challenges impacting response times “complex,” and extending beyond paramedic deployment into broader health system issues.

Call demand for paramedics is expected to keep rising at rates between two and five per cent a year as the city grows and the population ages, the service's statement said.

When few ambulances are available, the service says it does what it can to support patient safety, including having a patient safety advocate review calls that have been waiting “for an extended period” and doing call-backs to identify calls that might need reprioritization.

Purvis, Duncan's wife, looks back on that long September night waiting for help, and sees sharp contrasts. In Duncan's nine years with cancer, she said the in-hospital care he's received has been “incredible.” But on the frontlines of the healthcare system, she sees the cracks. She's seen people inside the emergency room who she believes could have been kept out if they could access earlier, preventative care, primary health services, even more basic needs like housing. “It just seems like emergency has sort of become this catch-all.”

That pressure mounts. And people, en masse, are left waiting.

A lot of what happened that night feels, in retrospect, like a blur – a screen her mind has thrown over a distressing, traumatic memory of calling back, over and over, and asking: where were the paramedics? When would they be able to get there and help her husband? It was a stress that only lifted when the first team of paramedics finally pulled up outside.

“Once they got here, I certainly felt a huge amount of relief.”